Gender-Inclusive Planning for Healthy Communities: Evidence from Three Neighbourhood Types in Guangzhou

Condensed English summary of my undergraduate thesis (original in Chinese, B.Sc. in Human Geography and Urban-Rural Planning, School of Geography and Planning, Sun Yat-sen University, May 2021; supervisor: Prof. Yuan Yuan). The full Chinese text is available on request.

Abstract

Planning in China's cities has long assumed a gender-neutral resident, while women carry most of the care work and spend the most time in their neighbourhoods. This study asks how neighbourhood environments support or constrain women's physical, mental and social health, and how those conditions differ across neighbourhood types. Drawing on site observation and in-depth interviews with 30 women in three contrasting Guangzhou neighbourhoods — a former work-unit (*danwei*) estate in the old city, an urban village and a commodity-housing estate — it finds that self-rated physical and mental health were broadly similar across sites, but social health differed sharply, even within the same neighbourhood. Social connection tracked length of residence and command of Cantonese, separating long-term locals from migrant tenants. The problems women reported cluster in five domains: environmental quality, streets and mobility, services and facilities, housing, and participation. Their form depended on each neighbourhood's age, planning history and resident mix. The thesis proposes differentiated strategies for each neighbourhood type rather than a single women-friendly template.

Keywords: gender-inclusive planning, healthy communities, women's health, neighbourhood types, Guangzhou

1. Background and research questions

China's *Healthy China 2030* plan names women as a group with specific health needs and treats the community as the place where health policy is delivered. Yet neighbourhood planning has rarely asked who its default user is. Age-friendly and child-friendly design have entered planning practice; gender has not.

Women are both the heaviest users and the main managers of neighbourhood space. In Guangzhou, women held 63.29% of seats on urban residents' committees and 32.3% on village committees. They also do most of the unpaid care work that keeps them close to home. When a neighbourhood lacks childcare, has fenced entrances a pram cannot pass, or has unlit lanes, the cost falls disproportionately on them.

The study asked three questions:

1. What health needs arise from women's bodies, care roles and everyday routines, and what do they require of the neighbourhood?
2. How do women in different neighbourhood types experience their environment and their own health?
3. What planning strategies would make each type of neighbourhood more gender-inclusive?

2. Conceptual framework

The study combines feminist geography with the healthy-communities literature. From feminist planning it takes the principle of *equality through difference*: planning should judge, case by case, whether a space is best designed universally or with gender-specific needs in mind. It also follows the argument that a women-friendly city is a precondition for an inclusive city. From the WHO definition, it treats health as three linked dimensions: physical, mental and social.

A bibliometric review (CiteSpace; 294 Web of Science papers and 107 CNKI papers) showed that the international field had moved since about 2014 from ideological debate toward physical activity and spatial strategy. It also showed that women's health responds more strongly than men's to the social side of the neighbourhood, such as trust, safety and ties. Responses also vary widely *among* women by education, employment and income. Four precedents informed the strategies: gender mainstreaming in Vienna's Frauen-Werk-Stadt, Barcelona's gender-equality and superblock programme, Ireland's work on women's travel patterns, and Victoria's design guidelines for women-friendly sports facilities.

From this review the study derived four principles for a gender-inclusive healthy community:

Principle	What it responds to	Domains where it matters most
Safety	Women's higher exposure to harassment and street crime, and the fear that limits their use of space	Streets and mobility; environmental quality
Convenience	Care work, trip-chaining, pregnancy and post-partum needs, gaps in women's health services	Services and facilities; housing
Liveability	Light, air, noise and cleanliness in everyday surroundings	Environmental quality; housing
Belonging	Legible, familiar spaces and a voice in how the neighbourhood is run	Participation; environmental quality

The five domains are not equally gendered. Housing and environmental quality mostly concern needs everyone shares, such as daylight and ventilation. Streets, services and participation are bound up with safety, the balance of paid and care work, and women's voice. Getting these wrong harms women most, so they need gender-specific planning.

3. Study design and methods

The three sites were chosen from a social-area typology of Guangzhou built from 2010 census data through factorial ecology. Each represents a major neighbourhood type with its own building age, planning history, service provision and resident mix.

Site	Type	Key characteristics
Zhujiangyuan (Yuexiu)	Former work-unit (*danwei*) estate, old city	9 ha, about 4,200 residents in 709 households; built for a state shipping company; older part (1985) unplanned and walled off from a newer part; mix of

Site	Type	Key characteristics
		ageing owners and renters; 5–6 storey walk-ups with no lifts
Hengsha (Huangpu)	Urban village under redevelopment	3.8 km²; 6,303 registered residents and 10,850 migrants; village dating to the Song dynasty; fishbone lanes 1.2–2 m wide; dense self-built "handshake" buildings; partial demolition since 2010
Meilin Haian (Tianhe)	Commodity-housing estate	34 ha, 3,128 households; built 2005; gated, with large water gardens and river frontage; many professional women who hire domestic help online

Four methods were combined:

- **Literature review and bibliometrics** to define the key concepts and the evidence on neighbourhood effects on women's health.
- **Systematic site observation** of green space, streets, lighting and facilities, and of how residents used them.
- **In-depth interviews** with 30 women, plus a few men for contrast. Participants were sampled across age (young, middle-aged, older), employment (working, retired, full-time homemaker) and household type (single, married without children, mothers, divorced). Interviews were treated as a joint construction of meaning: participants reflected on their health and daily life rather than simply reporting it.
- **Comparative analysis** across sites and across subgroups of women, to capture differences *among* women as well as between women and men.

Health was measured by self-rating: physical and mental health, and social health as felt closeness to neighbours.

4. Findings

4.1 Similar bodies, divided social lives

Most women rated their physical and mental health as good or very good, and the two ratings usually matched. Lower physical ratings came mainly from women in service jobs in the neighbourhood, such as cleaners and a hairdresser with chronic neck pain. Lower mental ratings came from women kept awake by noise and from new mothers.

Social health was the exception. Felt closeness to neighbours varied widely, even inside one neighbourhood. It depended mainly on how long a woman had lived there and whether she spoke Cantonese. In practice this separated long-term local residents from migrant tenants. The divide was sharpest in the urban village, where villagers, young migrant students and older tenants lived side by side with little contact.

Each neighbourhood type also housed a different group of women. The *danwei* estate was home mainly to retired women who had moved in to care for grandchildren. The urban village mixed villagers of all ages with low-income tenants. The commodity estate housed mainly working professionals who outsourced care work.

4.2 Problems by neighbourhood type

Zhujiangyuan (old *danwei* estate): decline of ageing infrastructure. Street lights had largely failed, leaving lanes dark at night. Broken paving and missing manhole covers made walking hazardous, especially with a pram. There was no play space where mothers could watch children while doing housework. A residents' committee home-help service had been discontinued, leaving elder and child care to women. Walk-up blocks without lifts meant carrying shopping and prams up five or six floors. One resident's complaint to the committee went unresolved for so long that she concluded it was "no use going."

Hengsha (urban village): missing basics and pervasive insecurity. Theft and street robbery were common. One woman had a necklace snatched and no longer wears jewellery outside. Lanes were unlit, full of dead ends and without signage; a tenant who had lived there a year still "could not find the way." Tightly packed buildings blocked daylight. Traffic, night-time trade and demolition noise disrupted sleep, most of all for women caring for infants. Women's health services were hard to reach, and most migrant women had never used the services offered by the residents' committee or the local Women's Home.

Meilin Haian (commodity estate): gender-neutral design that overlooks women. The basic infrastructure worked well, but details did not. Paving gaps caught heels, and there were many steps but few ramps for prams. Facilities centred on mahjong tables and seldom-used equipment for older residents, with no gym or exercise studio for working women. There was no on-site domestic or childcare service, so residents relied on online platforms with delays and trust risks. Shared corridor space in residential blocks went unused, and women described their neighbours as strangers.

5. Planning strategies

The thesis argues that a single women-friendly template would fail. Each neighbourhood type needs strategies matched to its core problem and its residents.

Domain	Old *danwei* estate (Zhujiangyuan)	Urban village (Hengsha)	Commodity estate (Meilin Haian)
Core problem	Ageing facilities	Missing basics; insecurity	Higher-level needs of working women unmet
Environmental quality	Repair street lighting; turn front-yard planting into usable social space	Wayfinding landmarks drawing on village heritage (ancestral halls, ponds, banyans); more greenery; lit, supervised pedestrian routes	Distributed bins; pet-waste enforcement
Streets and	Repair broken,	Lit bus stops; traffic calming; wider footpaths; active ground	More frequent, closer-spaced bus stops for

Domain	Old *danwei* estate (Zhujiangyuan)	Urban village (Hengsha)	Commodity estate (Meilin Haian)
mobility	uneven paving	floors and corner mirrors for natural surveillance	short multi-stop trips; ramps alongside steps
Services and facilities	Children's play areas within sight of homes; elder-care services; women's health outreach at the community clinic	Children's play space; accessible women's and maternal health services	Exercise space suited to working women; on-site cleaning, childcare and elder-care services
Housing	Add lifts to walk-ups of four or more storeys, with negotiation among owners	Phased removal of the most hazardous illegal buildings, with resettlement and protection for owners	Turn shared corridor space into places to meet and help each other
Participation	Public board showing the status of residents' complaints	Online resident groups and regular in-person forums; involve the Women's Federation and Women's Home	Stronger Women's Federation role in estate management

Two principles run across all three types. First, involve women and women's organisations from the start of any renewal, not only at consultation. Second, build lasting feedback mechanisms so that maintenance and responsiveness continue after the project ends. The thesis also argues that bringing domestic services into the neighbourhood turns unpaid care work into paid work. This both relieves women who work and creates jobs, many of them held by women.

The aim is not to keep women in the home or to push them out of it. A healthy community should support a woman who works outside the home and one who chooses full-time care, and let each take part in how the neighbourhood is run.

6. Limitations and reflection

Limitations noted in the thesis

- Health was self-rated. Privacy rules kept clinical data from community health centres out of reach, so ratings reflect perception as well as condition. The thesis called for routine collection of sex-disaggregated data in planning.
- The study looked only at the residential neighbourhood. Women's health is also shaped by workplaces and commutes, which need larger-scale data.
- Residents knew and trusted community clinics little and preferred city hospitals, which limited what the interviews could reveal about local health care.

Looking back

The sample was small and purposive, and the study was exploratory; its findings point to patterns rather than prove them. Some of the framing, drawn from the planning literature of the time, also leaned on generalisations about women as a group. I would now ground it more firmly in care roles, constraints and residents' own accounts.

The finding that stayed with me was not about women and men. It was the gap *among* women: in the same neighbourhood, a long-term Cantonese-speaking resident and a migrant tenant lived in effectively different social worlds. Who belongs, who is connected and who is left out turned out to depend on origin, tenure and language as much as gender. Inclusion in planning cannot be designed for a single group; it has to address how space sorts and separates people. That question — how urban space produces and reproduces social segregation — is what I went on to study.

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The full reference list (45 items) is in the original thesis.